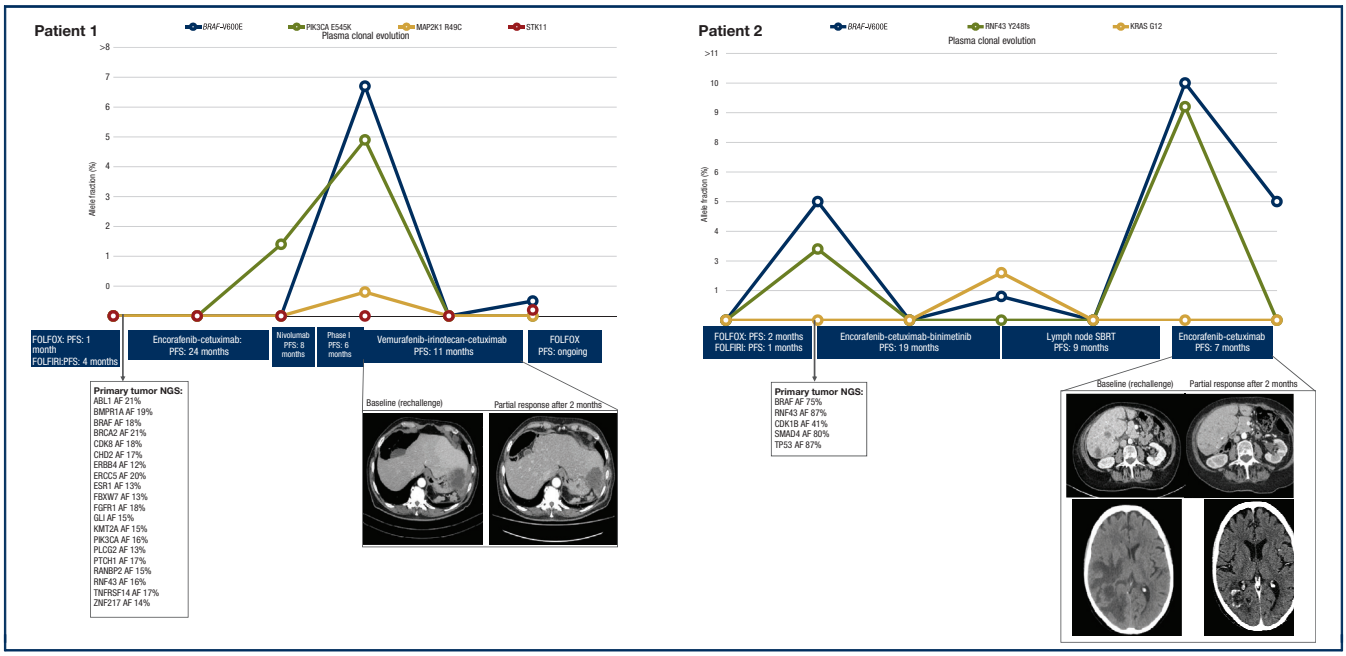

Figure 1. Clinical course, treatment PFS, key molecular findings, and allele fraction evolution from patient 1 and patient 2.
NGS, next-generation sequencing; PFS, progression-free survival; SBRT, stereotactic body radiation therapy.

Biopharma, AstraZeneca, Bayer, Boehringer Ingelheim, Chugai, Daiichi Sankyo, F. Hoffmann-La Roche Ltd, Genentech Inc, HalioDX SAS, Hutchison MediPharma International, Ikena Oncology, Inspirna Inc, IQVIA, Lilly, Menarini, Merck Serono, Merus, MSD, Mirati, Neophore, Novartis, Ona Therapeutics, Orion Biotechnology, Peptomyc, Pfizer, Pierre Fabre, Samsung Bioepis, Sanofi, Scandion Oncology, Scorpion Therapeutics, Seattle Genetics, Servier, Sotio Biotech, Taiho, Tessa Therapeutics, and TheraMyc. Stocks: Oniria Therapeutics and also educational collaboration with Imedex/HMP, Medscape Education, MJH Life Sciences, PeerView Institute for Medical Education and Physicians Education Resource (PER). AV declares to be stockholder of Reveal Genomics, consultants of Reveal Genomics, reports personal fees from Bayer, personal fees from Bristol Myers Squibb, personal fees from Guardant Health, personal fees from Merck, personal fees from Novartis, personal fees from Roche, personal fees from Incyte, and has a patent O2015145388A3 licensed. EE has received personal speaker honoraria from Organon and Novartis; and personal advisory board honoraria from Amgen, Bayer, Hoffman-La Roche, Merck Serono, Sanofi, Pierre Fabre, MSD, and Servier. Her institution has received research funding from Amgen Inc., Array Biopharma Inc., AstraZeneca Pharmaceuticals LP, BeiGene, Boehringer Ingelheim, Bristol Myers Squibb, Celgene, Debiopharm International SA, F. Hoffmann-La Roche Ltd., Genentech Inc, HalioDX SAS, Hutchison MediPharma International, Janssen-Cilag SA, MedImmune, Menarini, Merck Health KGAA, Merck Sharp & Dohme, Merus NV, Mirati, Novartis Farmacéutica SA, Pfizer, Pharma Mar, Sanofi Aventis Recherche & Développement, Servier, Taiho Pharma USA Inc. She held/holds non-remunerated roles as Coordinator of the SEOM +MIR Section of Residents and Young Assistants of the Sociedad Española de Oncología Médica (SEOM), speaker of the ESMO Academy of the European Society for Medical Oncology (ESMO), and volunteer member of the ASCO Annual Meeting Scientific Program Committee: Developmental Therapeutics — Immunotherapy of the American Society of Clinical Oncology (ASCO).

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Updated treatment recommendation for third-line treatment in advanced colorectal cancer from the ESMO Metastatic Colorectal Cancer Living Guideline

The following European Society for Medical Oncology (ESMO) Living Guideline has been recently updated with new treatment recommendations: ESMO Metastatic Colorectal Cancer Living Guideline.[1,2]

LIVING GUIDELINE UPDATE

View the ESMO Metastatic Colorectal Cancer Living Guideline here: https://www.esmo.org/living-guidelines/esmo-metastatic-colorectal-cancer-living-guideline.

Use of trifluridine—tipiracil—bevacizumab as third-line treatment for metastatic colorectal cancer

In the SUNLIGHT phase III trial, trifluridine—tipiracil—bevacizumab improved both progression-free survival [hazard ratio (HR) 0.44, 95% confidence interval (CI) 0.36-0.54, $P < 0.001$] and overall survival (HR 0.61, 95% CI 0.49-0.77, $P < 0.001$) in patients with metastatic colorectal cancer (mCRC) over monotherapy with trifluridine—tipiracil.[3] Patients included in this study had received no more than two previous chemotherapy regimens for the treatment of mCRC and had progressive disease or had experienced unacceptable adverse effects (AEs) from their most recent regimen. Previous treatment must have included a fluoropyrimidine, irinotecan, oxaliplatin, an anti-vascular endothelial growth factor monoclonal antibody (mAb) or an anti-epidermal growth factor receptor mAb, and the treatment could have included neoadjuvant or adjuvant chemotherapy if disease had recurred during treatment or within 6 months after the last administration of neoadjuvant or adjuvant therapy.

Severe AEs (grade ≥3) were reported in 72.4% of the patients in the trifluridine—tipiracil—bevacizumab group and in 69.5% of the patients in the trifluridine—tipiracil group. The most common AEs in both groups were neutropenia, nausea and anaemia. AEs that were more frequent in the trifluridine—tipiracil—bevacizumab group than in the trifluridine—tipiracil group were hypertension (10.2% versus 2.0%), nausea (37.0% versus 27.2%) and neutropenia (62.2% versus 51.2%), including severe (grade ≥3) neutropenia (43.1% versus 32.1%, respectively). AEs leading to discontinuation of treatment were similar in both arms of the trial. No treatment-related deaths were reported. Furthermore, Eastern Cooperative Oncology Group performance status deterioration was 9.3 months in the trifluridine—tipiracil—bevacizumab group and 6.3 months in the trifluridine—tipiracil group (HR 0.54, 95% CI 0.43-0.67). Trifluridine—tipiracil—bevacizumab was recently approved by the European Medicines Agency (EMA) and obtained an ESMO-Magnitude of Clinical Benefit Scale (ESMO-MCBS) v1.1 score of 4, which indicates substantial clinical benefit.

Figure 1 provides an updated treatment algorithm.

Recommendation

- Trifluridine—tipiracil—bevacizumab is recommended in patients pretreated with fluoropyrimidines, oxaliplatin, irinotecan and biologics, or in earlier lines of therapy following oxaliplatin and irinotecan regimen failure [I, A; ESMO-MCBS v1.1 score: 4].

METHODOLOGY

ESMO-MCBS v1.1[4] was used to calculate scores for new therapies/indications approved by the EMA or Food and Drug Administration (https://www.esmo.org/Guidelines/

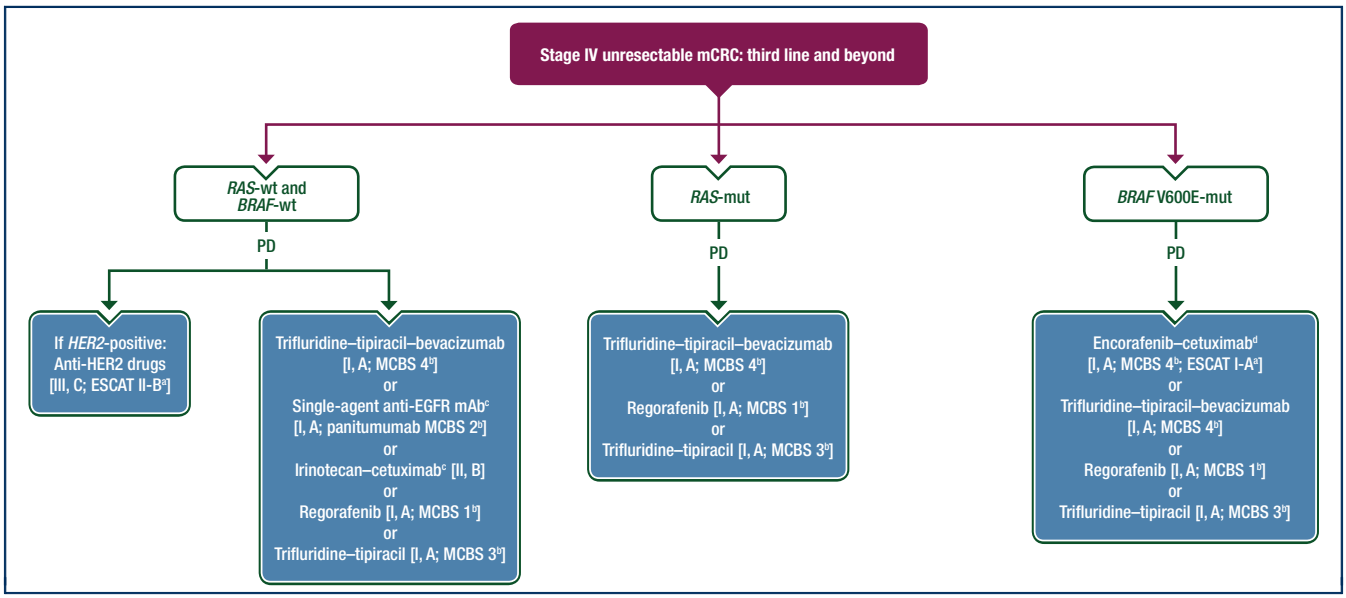

Figure 1. Stage IV unresectable mCRC: third-line treatment and beyond.
Purple: general categories or stratification; blue: systemic anticancer therapy; white: other aspects of management.
EGFR, epidermal growth factor receptor; EMA, European Medicines Agency; ESCAT, ESMO Scale for Clinical Actionability of Molecular Targets; FDA, Food and Drug Administration; HER2, human epidermal growth factor receptor 2; mAb, monoclonal antibody; MCBS, ESMO-Magnitude of Clinical Benefit Scale; mCRC, metastatic colorectal cancer; mut, mutant; PD, progressive disease; wt, wild-type.
[a]ESCAT scores apply to genomic alterations only. These scores have been defined by the guideline authors and validated by the ESMO Translational Research and Precision Medicine Working Group.[5]
[b]ESMO-MCBS v1.1[4] was used to calculate scores for therapies/indications approved by the EMA or FDA. The scores have been calculated and validated by the ESMO-MCBS Working Group and reviewed by the authors (https://www.esmo.org/guidelines/esmo-mcbs/esmo-mcbs-evaluation-forms).
[c]In *RAS*-wt patients not previously treated with anti-EGFR monoclonal antibodies.
[d]Treatment for *BRAF*-mut patients if not used in the second line.

ESMO-MCBS). The scores have been calculated and validated by the ESMO-MCBS Working Group and reviewed by the authors.

A. Cervantes[1,2] & E. Martinelli[3], on behalf of the ESMO Guidelines Committee*

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DISCLOSURE

AC reports fees paid to his institution as an invited speaker from Amgen, Foundation Medicine, Merck Serono and Roche; fees paid to his institution for advisory board membership from Amgen, AnHeart Therapeutics, Merck Serono, Roche and Transgene; personal fees for editorial roles as Associate Editor for *Annals of Oncology* and *ESMO Open* and as Editor for *Cancer Treatment Reviews*; institutional funding as principal investigator from Actuate Therapeutic, Adaptimmune, Amcure, Amgen, Astellas, AstraZeneca, Bayer, BeiGene, Bristol-Myers Squibb, FibroGen, Genentech, Lilly, MedImmune, Merck Serono, Merck Sharp & Dohme (MSD), Natera, Novartis, Servier, Sierra Oncology and Takeda; he reports a non-remunerated role as General and Scientific Director of INCLIVA Biomedical Research Institute. EM reports personal fees as an invited speaker from Bayer, ESMO, Incyte, Merck S.p.A., Merck Serono, Pierre Fabre, Roche and Servier; personal fees for writing engagements from AstraZeneca, Incyte, MSD, Pierre Fabre, Roche and Servier; personal fees for advisory board membership from Amgen, Incyte, MSD, Pierre Fabre, Roche and Servier.

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